

Riverside General Hospital

Comprehensive Policy and Procedure Manual

This manual is a fictional, synthetic document created for software testing purposes only. It does not represent the policies of any real hospital and contains no real patient data.

Chapter 1: General Patient Visiting Policy (Section 1)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 5: Fire Safety and Emergency Evacuation

(Section 1)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

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Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 1)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 1)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 1)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 1)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 1)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 1)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 1)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation

(Section 1)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization

(Section 1)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 1)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation

(Section 1)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 1)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

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High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

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Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization

(Section 2)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 2)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation

(Section 2)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 2)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 3)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 3)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 3)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 3)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 3)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 3)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 3)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 3)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

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In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 4)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 4)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 4)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 4)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 4)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 4)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 4)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 4)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 4)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 4)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 4)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

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Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

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Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 5)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 5)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 5)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 5)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 5)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 5)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 5)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 6)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 6)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 6)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 6)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

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Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 7)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 7)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 7)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 7)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 7)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 7)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 7)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 7)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 7)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 7)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 7)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 7)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

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Chapter 10: Consent and Patient Rights (Section 8)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 8)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 8)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 8)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 8)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 8)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 8)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 8)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 8)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation

(Section 8)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 8)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 9)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 18: Complaints and Patient Feedback (Section 9)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 9)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 9)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 10)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 10)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 10)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 10)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 10)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 10)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 10)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 10)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 10)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

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Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 11)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 11)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 11)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 11)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 11)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 11)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 11)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 11)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 11)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 11)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 11)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 11)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

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Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

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Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 12)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 12)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 12)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 12)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 12)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 12)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 13)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 13)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 13)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 13)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 13)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 3: Patient Discharge Procedure (Section 14)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 14)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 14)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 14)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 14)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 14)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 14)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 14)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 14)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 14)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 14)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 14)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

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Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

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Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 15)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 15)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 15)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 15)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 15)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 15)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 15)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 15)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 15)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 15)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 16)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 16)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

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Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 16)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 17)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 17)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 17)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 17)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 17)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 17)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 17)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 17)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 17)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 17)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

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All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

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Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Chapter 7: Nursing Shift Handover (Section 18)

Formal nursing handover takes place three times daily, at 7:00 AM, 2:00 PM, and 9:00 PM. The outgoing nurse reviews each patient at the bedside with the incoming nurse whenever clinically appropriate.

Handover must cover, at minimum, the patient's current diagnosis, vital sign trends, pending investigations, medication changes, and any safety concerns such as fall risk or confusion.

Any deviation from the standard handover process, including an abbreviated handover during an emergency, must be documented in the nursing notes with the reason recorded.

The charge nurse is responsible for ensuring handover has been completed for every patient on the ward before the outgoing shift leaves the premises.

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Chapter 8: Patient Identification Protocol (Section 18)

Before administering any medication, performing any procedure, or collecting any specimen, staff must check at least two patient identifiers, typically the full name and date of birth, against the wristband and the order.

Verbal confirmation from the patient, where the patient is able to respond, is used as an additional check alongside the wristband, but never as a replacement for it.

Patients who are unable to wear a wristband, such as those with severe burns, must have an alternative identification method documented at admission and clearly noted in the chart.

Any identification discrepancy, however minor, must be resolved before proceeding, and must be escalated to the charge nurse immediately.

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Chapter 9: Medication Administration Safety (Section 18)

Medications are administered following the five rights: right patient, right drug, right dose, right route, and right time, each independently verified against the medication administration record.

High-alert medications, including concentrated electrolytes and anticoagulants, require an independent double-check by a second qualified nurse before administration.

Any medication error, near miss, or adverse reaction must be reported through the hospital's incident reporting system within the same shift, regardless of whether harm occurred.

Verbal medication orders are accepted only in emergencies and must be read back to the prescriber for confirmation before being documented and countersigned within twenty-four hours.

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Chapter 10: Consent and Patient Rights (Section 18)

Written informed consent is required before any surgical procedure, invasive investigation, or treatment carrying significant risk, and must be obtained by a clinician capable of performing the procedure.

Patients have the right to receive information about their diagnosis, proposed treatment, alternatives, and expected outcomes in a manner they can understand, including through an interpreter where needed.

A patient with decision-making capacity has the right to refuse any treatment, and this refusal, along with the counselling provided, must be documented in the medical record.

For patients who lack decision-making capacity, consent is sought from the legally recognized next of kin or guardian in accordance with applicable local regulations.

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Chapter 11: Isolation and Barrier Precautions (Section 18)

Patients with suspected or confirmed airborne infections are placed in a negative-pressure isolation room whenever one is available, with signage posted clearly at the entrance.

Staff entering a contact-precaution room must don a gown and gloves before entry and remove them, followed by hand hygiene, immediately before leaving the room.

Visitors to an isolation room are limited to essential caregivers only and must be instructed in the correct use of personal protective equipment by the assigned nurse before entering.

Isolation status is reviewed daily by the treating team and downgraded as soon as clinically appropriate, with the change communicated to housekeeping and dietary services.

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Chapter 12: Blood Transfusion Safety (Section 18)

Every blood product must be verified at the bedside by two qualified staff members, cross-checking the patient's identity, blood group, and unit number against the compatibility label.

Vital signs must be recorded immediately before transfusion, at fifteen minutes after starting, and hourly thereafter until completion, with any adverse reaction managed per the transfusion reaction protocol.

Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

Unused blood products must be returned to the blood bank within the time limit specified on the issue slip and must never be re-stored in a ward refrigerator.

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Chapter 13: Pressure Injury Prevention (Section 18)

All patients are assessed for pressure injury risk within the first eight hours of admission using a validated risk assessment tool, and the result documented in the nursing care plan.

Patients identified as high risk are repositioned at least once every two hours, with the schedule documented on a repositioning chart kept at the bedside.

Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

Any pressure injury identified during a shift must be staged, photographed with consent, and reported through the incident reporting system the same day.

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Chapter 14: Falls Prevention Program (Section 18)

Every patient is screened for fall risk at admission and again after any significant change in condition, medication, or mobility status.

High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Chapter 15: Laboratory Specimen Collection (Section 18)

Specimen labels must be generated and applied at the patient's bedside immediately after collection, never in advance, to avoid mislabeling.

Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 18)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 18)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 18)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 18)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

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Any suspected transfusion reaction requires the transfusion to be stopped immediately, the intravenous line kept open with saline, and the blood bank notified without delay.

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Pressure-relieving mattresses and heel protectors are allocated according to the patient's assessed risk level and reassessed whenever the patient's condition changes.

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High-risk patients are identified with a designated wristband color and a bed-space sign, and are placed as close to the nursing station as ward layout allows.

Bed alarms, non-slip footwear, and adequate lighting are provided for all patients assessed as high risk, along with an individualized mobility plan agreed with physiotherapy.

Following any fall, the patient is assessed for injury immediately, vital signs are monitored per protocol, and the incident is reviewed at the next multidisciplinary huddle.

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Two patient identifiers must be verified against the specimen label and the request form before the sample is sent to the laboratory.

Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

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Chapter 16: Radiology Safety and Patient Preparation (Section 19)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

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Chapter 17: Employee Health and Immunization (Section 19)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

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Chapter 18: Complaints and Patient Feedback (Section 19)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Chapter 19: Medical Records and Documentation (Section 19)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Chapter 20: Emergency Department Triage (Section 19)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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Chapter 1: General Patient Visiting Policy (Section 20)

Intensive Care Unit visiting hours are from 4:00 PM to 6:00 PM daily. A maximum of two visitors are permitted at the bedside at any one time. Visitors under the age of twelve are not permitted in the Intensive Care Unit or the Isolation Ward without prior written approval from the attending physician.

General ward visiting hours are from 10:00 AM to 12:00 noon and again from 5:00 PM to 7:00 PM. Visitors are asked to check in at the nursing station before proceeding to the patient's room, and to use the alcohol-based hand rub provided at every ward entrance.

During declared outbreak periods, the hospital administration reserves the right to restrict visiting hours further, and such changes will be posted at every entrance and communicated to registered next of kin by phone.

Visitors who show signs of fever, cough, or other communicable symptoms will be asked to postpone their visit and may be offered a video call with the patient instead, arranged through the nursing station.

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Chapter 2: Hand Hygiene and Infection Prevention (Section 20)

All clinical staff must perform hand hygiene at five defined moments: before touching a patient, before any clean or aseptic procedure, after exposure to body fluid, after touching a patient, and after touching the patient's immediate surroundings.

When hands are visibly soiled, they must be washed with soap and water for a minimum of forty seconds. When hands are not visibly soiled, an alcohol-based hand rub applied for at least twenty seconds is the preferred method.

Gloves are not a substitute for hand hygiene. Hands must be cleaned immediately before donning gloves and immediately after their removal, regardless of whether the gloves appeared to be damaged.

Compliance audits are conducted monthly by the Infection Prevention and Control team, and results are displayed on ward noticeboards as part of the hospital's ongoing quality improvement program.

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Chapter 3: Patient Discharge Procedure (Section 20)

Discharge planning begins on the day of admission. The nurse in charge, in coordination with the treating physician, confirms an expected discharge date within the first twenty-four hours and updates it as the clinical picture evolves.

On the planned day of discharge, the billing desk prepares the final itemized bill, and the pharmacy dispenses take-home medication along with written dosing instructions in the patient's preferred language where available.

A patient who chooses to leave against medical advice must be counselled on the associated risks by the attending physician and must sign a formal declaration form, a copy of which is retained in the medical record.

Follow-up appointments, where clinically indicated, are booked before the patient leaves the ward, and a printed summary of the admission, diagnosis, and follow-up plan is provided at the time of departure.

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Chapter 4: Equipment Maintenance and Biomedical Support (Section 20)

Any staff member identifying a malfunctioning piece of equipment must submit a maintenance request form to the Biomedical Engineering department within the same shift, describing the fault as precisely as possible.

Breakdowns involving life-support equipment, including ventilators and infusion pumps, must be reported immediately by phone to the Biomedical on-call desk, in addition to the written request.

Equipment identified as faulty must be tagged 'Out of Service' and physically removed from patient-ready areas until a qualified biomedical engineer completes a documented safety inspection.

Preventive maintenance schedules are maintained centrally, and any equipment overdue for scheduled servicing is automatically flagged and withdrawn from circulation until the service is completed.

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Chapter 5: Fire Safety and Emergency Evacuation (Section 20)

In the event of a fire, staff follow the RACE protocol: Rescue anyone in immediate danger, Raise the alarm using the nearest call point, Contain the fire by closing doors and windows, and Extinguish it only if it is safe to do so using the nearest appropriate extinguisher.

Lifts must never be used during a fire alarm. All staff, patients able to walk, and visitors must proceed to the nearest marked staircase and assembly point.

Patients who are unable to be moved independently are evacuated according to the ward's horizontal evacuation plan, which relocates them behind the nearest fire-resistant compartment door before any vertical evacuation is attempted.

Fire drills are conducted in every department at least once every six months, and attendance is recorded by the departmental fire warden and reviewed by the hospital's Safety Committee.

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Chapter 6: Sharps and Clinical Waste Disposal (Section 20)

Used needles, blades, and other sharps must be disposed of directly into a puncture-resistant sharps container at the point of use. Needles must never be recapped, bent, or broken by hand.

Sharps containers must be sealed and replaced once they reach three-quarters of their capacity, and must never be filled beyond the manufacturer's indicated fill line.

Collection of sealed sharps containers and other clinical waste is performed by the housekeeping team each evening, following the color-coded waste segregation policy displayed in every clinical area.

Any needlestick or sharps injury must be reported immediately to the shift supervisor and to Occupational Health, and first-aid and post-exposure protocols must be initiated without delay.

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Specimens requiring time-critical processing, such as arterial blood gases, must be transported to the laboratory within the timeframe specified in the laboratory handbook.

Any rejected specimen is reported back to the requesting ward with the reason for rejection, and a repeat sample is arranged as clinically indicated.

Additional note 1: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Additional note 2: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Additional note 3: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Chapter 16: Radiology Safety and Patient Preparation (Section 20)

All female patients of childbearing age must be asked about the possibility of pregnancy before any imaging involving ionizing radiation, and the response documented on the request form.

Patients with implanted devices, such as pacemakers, must be screened using the MRI safety questionnaire before being scheduled for magnetic resonance imaging.

Contrast media are administered only after checking for known allergies and renal function, and emergency equipment must be immediately available in the imaging suite during contrast studies.

Radiology staff verify the imaging request against the clinical indication and patient identity before proceeding, escalating any mismatch to the requesting physician.

Additional note 1: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

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Additional note 3: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Chapter 17: Employee Health and Immunization (Section 20)

All clinical staff are required to complete the hospital's immunization program, including annual influenza vaccination, as a condition of continued clinical duties.

Staff exposed to a needlestick injury or bloodborne pathogen must report to Occupational Health within one hour for risk assessment and post-exposure prophylaxis where indicated.

Staff experiencing symptoms of a communicable illness are required to notify their supervisor before the start of their shift and to follow the hospital's fitness-for-duty guidance.

Annual health checks, including fitness for respiratory protective equipment, are coordinated by Occupational Health and scheduled with minimal disruption to ward staffing.

Additional note 1: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

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Additional note 3: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Chapter 18: Complaints and Patient Feedback (Section 20)

Patients and families may lodge a complaint verbally with any staff member or in writing through the Patient Relations office, and every complaint is acknowledged within two working days.

Complaints involving potential harm are escalated immediately to the relevant department head and reviewed through the hospital's incident management process.

The Patient Relations office provides a written response outlining the findings and any corrective action within fifteen working days of receiving the complaint.

Anonymized themes from complaints and compliments are reviewed quarterly by the Quality Committee to identify opportunities for service improvement.

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Additional note 2: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

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Chapter 19: Medical Records and Documentation (Section 20)

All clinical entries must be timed, dated, and signed by the person making the entry, with printed name and designation, in accordance with the hospital's documentation standard.

Corrections to a written record are made with a single line through the error, initialed and dated, never by erasure or the use of correction fluid.

Access to electronic medical records is granted according to the principle of least privilege, and every access is logged and subject to periodic audit.

Medical records are retained for the period specified by applicable regulation and are securely destroyed thereafter following the hospital's data retention policy.

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Additional note 2: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Additional note 3: staff are reminded to consult the department-specific addendum to this chapter for local variations in procedure, and to direct any questions to the relevant department head or the Quality and Safety office.

Chapter 20: Emergency Department Triage (Section 20)

All patients presenting to the Emergency Department are triaged within ten minutes of arrival using a five-level acuity scale by a qualified triage nurse.

Patients triaged as immediately life-threatening are taken directly to the resuscitation area, bypassing the waiting room entirely.

Re-triage is performed for any patient whose condition changes while waiting, or automatically at intervals appropriate to their assigned acuity level.

Triage decisions, including the assigned acuity level and time, are documented in real time and are subject to periodic audit by the Emergency Department's quality lead.

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